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FOCUS GASTROENTEROLOGY

16 April 2026

Dr Christina Illing
Andrew Place Clinic

Dear Dr Illing,

Re: Miss Poppy Kennedy
DOB: 2006-10-20
Address: 6 Mari Close, GREENSBOROUGH, VIC, 3088

Summary

- **Persistent upper gastrointestinal symptoms despite pantoprazole therapy.**
- **For gastroscopy to evaluate for Crohn's-related oesophageal ulceration versus opportunistic infection.**
- **Normal faecal calprotectin indicating stable lower gastrointestinal disease.**
- **For intestinal ultrasound later this year.**
- **No action required by you.**

I had the pleasure of conducting a review with Poppy Kennedy. As you are aware, Poppy has a history of Crohn's disease with previous oesophageal involvement, currently managed with Steqyma. Our review focused on her recent recurrence of chest discomfort and epigastric pain.

Poppy has been taking pantoprazole daily before breakfast. While this initially provided some symptomatic relief, her symptoms have since returned. She describes chest pain and discomfort that often occurs when she is fasted. She denies any dysphagia or the sensation of food sticking, though she noted that her symptoms have been exacerbated during a recent period of increased stress.

Regarding her Crohn's disease, she reports some occasional abdominal pain and the passage of mucus. However, her recent faecal calprotectin was normal, which is reassuring and suggests there is no significant active inflammation in the lower gastrointestinal tract. Consequently, I do not believe a colonoscopy is required at this stage, although I intend to perform one in approximately 12

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320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

to 24 months to evaluate for deep mucosal healing. I have also scheduled an intestinal ultrasound for later this year as part of her ongoing surveillance.

Impression and Plan The persistence of Poppy's upper gastrointestinal symptoms despite acid suppression is concerning. Given her immunosuppressed status, we must differentiate between active Crohn's-related oesophageal ulcers and opportunistic infections, such as fungal or viral oesophagitis. I have therefore recommended an urgent gastroscopy to obtain definitive histological aetiology.

In the interim, Poppy will continue her daily pantoprazole. I have advised her to implement lifestyle modifications, including consuming her evening meal by 6:00 pm and taking Gaviscon (preferably the liquid formulation) prior to retiring at night to mitigate any reflux component. I have provided her with a repeat prescription for pantoprazole and will review her following her gastroscopy.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist